

LEGAL ETHICAL ISSUES IN PSYCHIATRIC NURSING



LEGAL AND ETHICAL CONSIDERATIONS

Rights of Clients

- Mental health clients retain all civil rights afforded to all people except the right to leave the hospital in the case of involuntary commitment



Involuntary Hospitalization

- Laws are determined by each state. Know the laws of the state where you practice.
- Persons detained in this way lose only the right to freedom; all other rights are intact.
- Persons held without their consent must present an imminent danger to themselves or others; this must be proven at a hearing if the person is to be committed



BOX 9.1 Highlights of Patient's Bill of Rights

- To be informed about benefits, qualifications of all providers, available treatment options, and appeals and grievance procedures
- Least restrictive environment to meet needs
- Confidentiality
- Choice of providers
- Treatment determined by professionals, not third-party payers
- Parity
- Nondiscrimination
- All benefits within scope of benefit plan
- Treatment that affords greatest protection and benefit
- Fair and valid treatment review processes
- Treating professionals and payers held accountable for any injury caused by gross incompetence, negligence, or clinically unjustified decisions



- Civil commitment or involuntary hospitalization curtails the
- client's right to freedom (the ability to leave the hospital when they wish)



Release From the Hospital

Clients hospitalized voluntarily have the right to request discharge at any time and must be released unless they represent a danger to themselves or others; if such a danger is present, then commitment proceedings must be instituted to keep them in the hospital.

Clients who are no longer dangerous must be discharged from the hospital.



Release From the Hospital

- Clients hospitalized against their will under these conditions are committed to a facility for psychiatric care until they no longer pose a danger to themselves or to anyone else.
- 48 to 72 hour – maximum hours for a patient to be detained in an emergency basis.



- Legally assisted or mandatory outpatient treatment is the requirement that clients continue to participate in treatment on an involuntary basis after their release from the hospital into the community.



Conservatorship and Guardianship

Legal guardianship is separate from civil commitment for hospitalization.

A hearing can be held to determine whether the person is competent. An incompetent client cannot provide his or her own shelter, food, and clothing, cannot act in his or her own best interests, and cannot run his or her own business and financial affairs. If found incompetent, a guardian is appointed to speak for the client.



An incompetent client can no longer:

Enter into legal contracts with a signature

Sign checks



Use a credit card

Make a will

Open bank accounts

Sell property

Get married

Give consent for surgery



Least Restrictive Environment

Treatment must be provided in the least restrictive environment appropriate to meet the client's needs.

This philosophy is central to the deinstitutionalization of large state hospitals and the move to community-based care and services.

Physical restraint or seclusion in a locked room can be used only when the person is imminently aggressive or threatening to harm self.



Restraint and seclusion, if used, must be in place for the shortest time necessary. Many regulations govern the monitoring of clients in seclusion or restraint for their safety.

Restraint and seclusion standards are being revised frequently.

Duty to Warn Third Parties

Duty to warn a third party is an exception to client confidentiality. Clinicians must warn identifiable third parties of threats made by a client.



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When making a decision about warning a third party, the clinician must base his or her decision on the following:

- • Is the client dangerous to others?
- • Is the danger the result of serious mental illness?
- • Is the danger serious?
- • Are the means to carry out the threat available?
- • Is the danger targeted at identifiable victims?
- • Is the victim accessible?



Insanity is a legal term (not medical) that means the person could not control his or her actions or understand the difference between right and wrong at the time of the crime (M'Naghten rule). If insane, the person can be found not guilty of the crime in most states.

Thirteen states have provisions for a 'guilty, but insane' verdict which holds the person responsible for the crime while ensuring that he or she receives treatment.



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- M’Naghten rule—the defendant did not know the nature or quality of the act or did not know it was wrong because of a mental disease or defect.
- Irresistible impulse—the defendant could not control their conduct owing to mental disease or defect.
- Substantial capacity test—the defendant lacks substantial (but not total) capacity to appreciate their conduct as wrong or cannot conform their conduct to the law.
- Durham—the defendant’s criminal conduct is excused when it is caused by a mental disease or defect



Nursing Liability

Nurses are responsible for providing safe, competent, legal, and ethical care to clients and families. Nurses are expected to meet standards of care, meaning care provided meets set expectations and is what any nurse would do in a similar situation.

A *tort* is a wrongful act that results in injury, loss or damage, and may be intentional or unintentional.

Negligence is an unintentional tort that involves harm caused by failure to do what is reasonable and prudent.



Malpractice is a type of negligence specifically applied to doctors and nurses; a successful malpractice suit must prove: duty to the client; breach of that duty; injury or damage to the client; breach of duty was the direct cause of the injury or loss.

Intentional torts include: *assault* (causes person to fear being touched in an offensive manner); *battery* (harmful or unwanted actual contact); *false imprisonment* (unjustifiable detention).

Nurses can minimize the risk of lawsuits through safe, competent nursing care and descriptive, accurate documentation.



BOX 9.2

Tarasoff Versus Regents of the University of California (1976)

In 1969, a graduate student at the University of California, Prosenjit Poddar, dated a young woman named Tatiana Tarasoff for a short time. After the brief relationship ended, Poddar sought counseling with a psychologist at the university. He confided to the therapist that he intended to kill his former girlfriend when she returned from Brazil at the end of the summer. The psychologist contacted the university campus police, who detained and questioned Poddar. He was released because he appeared rational, promised to stay away from Tarasoff, and claimed he would not harm her. Two months later, shortly after her return from Brazil, Tatiana Tarasoff was murdered by Poddar on October 27, 1969. Her parents sued the University of California, claiming that the

therapist had a duty to warn their daughter of Poddar's threats. The California Supreme Court concluded that protective privilege ends where public peril begins.



Unintentional Torts:

*harm happens
because of
carelessness or*

- **Four elements:**

- **Duty:** A legally recognized relationship (i.e., physician to client, nurse to client) existed. The nurse had a duty to the client, meaning that the nurse was acting in the capacity of a nurse.
- **Breach of duty:** The nurse (or physician) failed to conform to standards of care, thereby breaching or failing the existing duty. The nurse did not act as a reasonable, prudent nurse would have acted in similar circumstances.

negligence



- **Injury or damage:** The client suffered some type of loss, damage, or injury.
- **Causation:** The breach of duty was the direct cause of the loss, damage, or injury. In other words, the loss, damage, or injury would not have occurred if the nurse had acted in a reasonable, prudent manner.

caregiver or wife



Steps to Avoid Liability

- Practice within the scope of state laws and nurse practice act.
- Collaborate with colleagues to determine the best course of action.
- Use established practice standards to guide decisions and actions.
- Always put the client's rights and welfare first.
- Develop effective interpersonal relationships with clients and families.
- Accurately and thoroughly document all assessment data, treatments, interventions, and evaluations of the client's response to care.



Ethical Issues

Ethics is a branch of philosophy that deals with values of human conduct (rightness and wrongness of actions) and the goodness or badness of the motives and ends of such actions.

Utilitarianism is a theory that bases decisions on 'the greatest good for the greatest number'.

✓ *Do your duty - the rule*
Deontology says decisions should be based on whether an action is morally right or wrong, with no regard for the consequences.



Ethical Issues (cont'd)

Deontological principles include:

- *Autonomy* – right to self-determination and independence
- *Beneficence* – duty to benefit others or promote good
- *Nonmaleficence* – do no harm
- *Justice* – fairness
- *Veracity* – honesty, truthful
- *Fidelity* – honor commitments and contracts



Ethical Dilemmas in Mental Health

Ethical dilemma – a situation in which ethical principles conflict or there is no one clear course of action.

Many dilemmas in mental health involve the client's right to self-determination and independence (autonomy) and concern for the 'public good' (utilitarianism).

The ANA Code of Ethics for Nurses is a guide to ethical behavior for nurses.



Ethical Dilemmas in Mental Health (cont'd)

Models for ethical decision-making include gathering information, clarifying values, identifying options, identifying legal considerations and practical restraints, building consensus for the decision reached, and reviewing and analyzing the decision to determine what was learned.



Ethical Dilemmas in Mental Health (cont'd)

Self-awareness for nurses is essential so that nurse's own values do not interfere with care to clients and families, including the ability to be an advocate for the client.



Content Areas for Ethical Code

- Compassion, respect, human dignity, and worth
- Primary commitment to patients
- Promotion of health, safety, and patient rights
- Responsible, accountable provision of care
- Professional growth, integrity, and competence of the nurse
- Promotion of safe, ethical health care/work environment
- Advancement of the nursing profession
- Collaboration with others
- Maintain integrity of profession, include social justice



Points to Consider When Confronting Ethical Dilemmas

- Talk to colleagues or seek professional supervision. Usually, the nurse does not need to resolve an ethical dilemma alone.
- Spend time thinking about ethical issues, and determine what your values
- and beliefs are regarding situations before they occur.
- • Be willing to discuss ethical concerns with colleagues or managers. Being silent is condoning the behavior.



EVIDENCE-BASED PRACTICE: Readiness for Shared Decision-Making

Shared decision-making (SDM) may be difficult for some clients—that is, they are not yet ready to fully participate. These authors identified elements of client readiness, such as

understanding of SDM, health literacy, communication skills, self-awareness, and consideration skills. When clients aren't ready for SDM, providers need to address barriers to readiness and assist clients to move toward readiness for SDM.

Reference: Keij, S. M., van Duijn-Bakker, N., Stiggelbout, A. M., & Pieterse, A. H. (2021). What makes a patient ready for shared decision making: A qualitative study. Patient Education and Counseling, 104(3), 571–577. <https://doi.org/10.1016/j.pec.2020.08.031>



Thank you

